

NORTHGATE GENERAL HOSPITAL — DISCHARGE SUMMARY

Department of Internal Medicine & Cardiology

Patient: Margaret Elaine Forsythe | DOB: 14/03/1958 (Age 66) | MRN: NGH-2024-087341

Admission Date: 09 June 2024 Discharge Date: 14 June 2024 Ward: Cardiology East, Bed 7B

Admitting Consultant: Dr. A. Rashid (Cardiology) GP: Dr. P. Nkemelu, Oakfield Surgery

REASON FOR ADMISSION / PRESENTING COMPLAINT

Mrs Forsythe, a 66-year-old female with a background of type 2 diabetes mellitus, hypertension, and known ischaemic heart disease, presented to A&E on the evening of 09/06/2024 via ambulance following an episode of acute central chest pain radiating to the left arm, associated with diaphoresis and shortness of breath. Symptoms began approximately 45 minutes prior to arrival. Denies nausea or vomiting. No syncope. She reports similar but milder chest discomfort over the preceding 3 days which she attributed to 'indigestion'. On direct questioning, she admits to increasing exertional dyspnoea over the past 2 weeks — she can now only manage one flight of stairs before needing to rest (previously had no limitations).

PAST MEDICAL HISTORY

Type 2 Diabetes Mellitus (diagnosed 2009, on insulin since 2018). Hypertension — longstanding, poorly controlled per GP records. Ischaemic Heart Disease — previous MI in 2019, stented LAD at Northgate. Hypercholesterolaemia. CKD Stage 2 (last eGFR 68 in Feb 2024). Osteoarthritis bilateral knees. Appendectomy 1987.

MEDICATIONS ON ADMISSION (as reported by patient — reconciliation pending)

Metformin 500mg BD — NOTE: patient states she stopped this herself 'a few months ago' due to GI upset. Insulin Glargine (Lantus) 24 units at night. Ramipril 10mg OD. Amlodipine 10mg OD. Atorvastatin 80mg nocte. Aspirin 75mg OD. Clopidogrel 75mg OD. Bisoprolol 5mg OD (patient unsure if she is still taking this — 'I ran out last month'). Omeprazole 20mg OD (PRN — rarely takes it).

ALLERGIES / ADVERSE REACTIONS

PENICILLIN — anaphylaxis (documented). Codeine — causes severe nausea. Contrast dye (iodinated) — previous reaction, details unclear from patient, written up as 'allergy' in old notes from 2017.

INVESTIGATIONS

ECG on arrival: Sinus rhythm, rate 94 bpm. ST depression V4-V6. T-wave inversion leads I and aVL. No LBBB. Compared to previous ECG (2023) — new changes. Troponin I (high sensitivity): 142 ng/L at 0h, 389 ng/L at 3h (rising pattern, significant). BNP: 810 pg/mL (elevated, consistent with acute decompensation). FBC: Hb 10.9 g/dL (mild anaemia), WBC 9.1, Platelets 224. U&E: Na 138, K 4.1, Urea 9.4, Creatinine 118 umol/L (eGFR 52 — reduced from baseline). HbA1c: 9.2% — suboptimal glycaemic control. LFTs: normal. CXR: mild pulmonary oedema, no consolidation, heart size borderline enlarged. Echo (12/06): EF 38% (reduced from 52% in 2023 echo). Mild MR. Moderate LV impairment. Angiogram (13/06): In-stent restenosis LAD stent (70% stenosis). RCA — 40% proximal lesion, not flow-limiting. LCx — patent.

CLINICAL COURSE AND MANAGEMENT

Admitted under cardiology. Diagnosis of NSTEMI confirmed. Dual antiplatelet therapy continued. Commenced IV GTN, low molecular weight heparin (enoxaparin 1mg/kg BD — dose-adjusted for CKD). Cardiology team proceeded to coronary angiogram Day 4. Drug-eluting stent deployed to LAD in-stent restenosis site — technically successful procedure, no complications. Post-procedure she was pain-free. HF team reviewed in view of reduced EF — commenced Sacubitril-Valsartan 24/26 mg BD (Entresto) in place of ramipril. Bisoprolol uptitrated to 7.5 mg OD. Furosemide 40mg OD added for fluid overload. Anaemia worked up — iron studies suggest iron deficiency (ferritin 11), commenced oral ferrous sulphate 200mg BD. Diabetic team reviewed — Metformin WITHHELD given CKD and contrast use. Lantus dose adjusted to 28 units nightly. Dietary review organised. Patient educated re: medication adherence — she had not been taking Bisoprolol regularly.

DISCHARGE DIAGNOSIS

1. NSTEMI secondary to in-stent restenosis, LAD — treated with PCI and DES. 2. Reduced ejection fraction heart failure (EF 38%). 3. Type 2 Diabetes Mellitus — suboptimally controlled (HbA1c 9.2%). 4. Iron Deficiency Anaemia. 5. CKD Stage 3a (acute on chronic, eGFR 52, baseline 68).

DISCHARGE MEDICATIONS (CHANGES FROM ADMISSION — PLEASE REVIEW)

STOPPED: Ramipril (replaced by Entresto). Metformin (withheld — do not restart until eGFR > 60 and contrast cleared). CHANGED: Bisoprolol increased to 7.5mg OD. Insulin Glargine increased to 28 units nocte. ADDED: Sacubitril-Valsartan (Entresto) 24/26mg BD — titrate up at 4 weeks if tolerated. Furosemide 40mg OD. Ferrous Sulphate 200mg BD (with food). CONTINUED: Atorvastatin 80mg, Aspirin 75mg, Clopidogrel 75mg, Omeprazole 20mg OD (now regular, not PRN, given dual antiplatelets).

RED FLAGS / SAFETY NETTING

Return immediately to A&E if: recurrent chest pain, breathlessness at rest, severe dizziness or syncope, weight gain > 2kg in 24 hours (fluid overload sign), urine output significantly reduced. GP to check U&E and eGFR in 2 weeks — Entresto contraindicated if K > 5.4 or eGFR drops significantly. HbA1c to be rechecked in 3 months. Cardiology OPD in 6 weeks. HF Nurse Specialist to contact within 48 hours of discharge. NOTE: Patient lives alone — social circumstances reviewed, daughter flagged as next of kin.

MISSING / OUTSTANDING INFORMATION

Iodinated contrast allergy — exact nature and severity of 2017 reaction not documented in current records. GP to clarify and update allergy record. Bisoprolol adherence history unclear — patient self-reporting unreliable. Smoking history: patient states 'ex-smoker', cessation date not confirmed. No formal cognitive assessment performed despite mild confusion noted by nursing staff on Day 2 (may be post-procedural).